

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

Building Up: A Blueprint for the Alberta Accessibility and Inclusion Act

Part Three — The Alberta Accessibility and Inclusion Act: A Constructive Blueprint

Section 3.2 — Definitions, Scope, and Coverage: The Substantive Ground of the Legislation

Section 3.1 set out the design philosophy and ten foundational choices that anchor the *Alberta Accessibility and Inclusion Act*. Section 3.2 develops the definitions, scope, and coverage provisions that operationalize those choices in the legislation's substantive ground. Definitions, scope, and coverage are not technical preliminaries; they are dispositive of who the legislation protects, what counts as a barrier, which organizations are bound by the legislation's substantive standards, and which substantive areas the legislation regulates. The definitions chosen and the coverage thresholds set will determine the legislation's substantive reach for as long as it operates.

The section develops four substantive components. First, the definition of disability the legislation will adopt: the social-model definition oriented toward barrier removal rather than impairment characterization, designed to be more inclusive than the comparator definitions where the comparator definitions have been criticized as inadequate. Second, the definition of barrier: the comprehensive five-category definition that addresses the full range of obstacles persons with disabilities encounter, including the systemic and engineered barriers documented in Part One. Third, the coverage and scope provisions: which organizations the legislation regulates, with what substantive obligations, and on what implementation timelines, including the small-organization and Indigenous-governance accommodations the comparator record demonstrates are operationally workable. Fourth, the identification of the eight substantive-standards areas the legislation establishes, with brief substantive specifications for each (the detailed substantive content is developed in Sections 3.3 and 3.4).

The substantive choices Section 3.2 develops align with the design philosophy and foundational choices of Section 3.1. Where the comparator legislation has adopted definitions that the substantive-equality and systemic-transformation principles would now identify as insufficient, Section 3.2 specifies more inclusive alternatives. Where the comparator legislation has established coverage thresholds that have produced operational gaps, Section 3.2 specifies thresholds calibrated to address the gaps. The substantive ground is, accordingly, the operational expression of the design philosophy at the most fundamental level of the legislation's structure.

The Definition of Disability

The legislation must adopt a definition of disability that operates within the social model rather than within the medical model. The distinction is foundational. The medical model defines disability as a condition of the individual — a physical, mental, or sensory impairment that the individual *has* and that the system responds to through accommodation or specialized programming. The social model defines disability as the interaction between an individual's impairment and the barriers the social environment imposes — the

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

disability is in the interaction, not in the impairment alone. The social model is the substantive framework the United Nations *Convention on the Rights of Persons with Disabilities* operationalizes. It is the substantive framework the federal *Accessible Canada Act* adopts. It is the substantive framework four of the five comparator provincial accessibility statutes adopt in varying forms. The Alberta legislation must adopt the social-model framework explicitly and should specify the definition in terms more inclusive than the comparator definitions where the comparator definitions have been criticized as insufficient.

The proposed substantive definition is as follows:

"Disability" includes any physical, mental, intellectual, cognitive, learning, communication, or sensory impairment, and any chronic illness or functional limitation, whether the impairment, illness, or limitation is permanent, temporary, episodic, fluctuating, or progressive in nature, and whether or not the impairment, illness, or limitation is visible or apparent, that, in interaction with a barrier as defined in this Act, hinders the person's full and equal participation in society on an equal basis with others.

The proposed definition has five substantive characteristics that distinguish it from the narrower comparator definitions and that operationalize the design philosophy of Section 3.1.

Comprehensive impairment categories. The definition explicitly enumerates physical, mental, intellectual, cognitive, learning, communication, and sensory impairment categories, mirroring the ACA's enumeration. It also explicitly includes chronic illness and functional limitation as separate categories. The chronic-illness inclusion addresses populations whose disabilities are produced by ongoing health conditions (multiple sclerosis, lupus, fibromyalgia, long COVID, severe forms of common conditions like asthma or arthritis) but whose impairments may not be characterized in the impairment-category language of the medical model. The functional-limitation inclusion addresses populations whose limitations are real and disabling but whose impairment characterization is contested or uncertain. The substantive purpose of the comprehensive enumeration is to ensure that the legislation's protections operate for all persons whose participation is hindered by interaction with barriers, regardless of how the underlying condition is characterized in medical-model terms.

Explicit inclusion of episodic and fluctuating disability. The definition explicitly includes episodic, fluctuating, and progressive disabilities alongside permanent and temporary ones. The episodic and fluctuating inclusions are operationally significant. Persons whose disabilities operate episodically (mental-health conditions with periods of remission and relapse, multiple sclerosis with relapsing-remitting course, migraine, certain forms of epilepsy) or whose disabilities fluctuate (chronic pain conditions, chronic fatigue syndromes, dysautonomia) have historically been excluded from disability-policy protections that require demonstration of continuous incapacity. The substantive inclusion ensures that the legislation's protections operate during periods of impairment-driven limitation, regardless of whether the limitation operates continuously. The progressive inclusion ensures that the legislation's protections operate throughout the trajectory of degenerative conditions, not only after the conditions have produced demonstrable severe limitation. The substantive standard is interaction with a barrier, not characterization of the impairment.

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

Explicit inclusion of invisible disability. The definition explicitly includes impairments and conditions whether or not they are visible or apparent. The invisible-disability inclusion addresses populations whose disabilities are not externally apparent — many learning disabilities, many mental-health conditions, many neurodivergent conditions, many chronic illnesses, many chronic-pain conditions, many forms of cognitive impairment. The exclusion of invisible disabilities from operational disability protections has been the subject of substantial disability-community advocacy across Canadian jurisdictions. The substantive purpose of the explicit inclusion is to foreclose interpretations of the legislation that would limit its protections to externally apparent disabilities.

Substantive-equality framing. The definition's operative phrase — *hinders the person's full and equal participation in society on an equal basis with others* — mirrors the substantive-equality language of Article 1 of the United Nations *Convention on the Rights of Persons with Disabilities*. The framing operationalizes the substantive-equality principle of Section 3.1. The legislation's protections operate against any barrier that hinders participation on an equal basis, regardless of whether the barrier is the result of explicit discrimination or of facially neutral design that produces disproportionately negative outcomes.

Independence from program-eligibility definitions. The definition operates independently of the eligibility definitions used in Alberta's categorical-program architecture (AISH severity criteria, PDD developmental-disability criteria, FSCD eligibility criteria, ADAP categorical criteria, WCB injury and impairment criteria). A person whose impairment does not meet the AISH severity threshold is, on the proposed legislation's substantive definition, a person with a disability for purposes of the legislation if their impairment, in interaction with a barrier, hinders their participation. The substantive purpose of the independence is to ensure that the categorical-program architecture's gatekeeping function does not constrain the substantive-standards architecture's protective function. The legislation operates in the substantive-standards architecture (Section 2.8 distinction); the categorical-program architecture continues to operate in its income-and-support function with its own eligibility criteria; the legislation's protections are not limited to persons who qualify for the categorical programs.

The proposed definition is, on these five substantive characteristics, more inclusive than the federal *Accessible Canada Act* definition in the explicit inclusion of episodic and fluctuating disability and in the explicit inclusion of chronic illness as a separate category. It is more inclusive than several comparator provincial definitions in the explicit inclusion of invisible disability. It is more inclusive than every provincial categorical-program definition in its independence from program-eligibility criteria. The substantive choice is to operate the legislation's protective function at the broadest substantive scope consistent with the social-model framework and the international human rights framework.

The Definition of Barrier

The legislation must adopt a comprehensive definition of barrier that addresses the full range of obstacles persons with disabilities encounter. The definition is the operational counterpart to the disability definition. Where the disability definition specifies who the legislation protects, the barrier definition specifies what the legislation protects against. The two definitions operate together: the substantive-equality principle requires that the legislation protect persons with disabilities (broadly defined) against barriers (broadly

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

defined) that hinder participation on an equal basis with others.

The proposed substantive definition is as follows:

"Barrier" means anything that hinders the full and equal participation of a person with a disability in society, including but not limited to:

(a) physical and architectural barriers, including any element of the built environment, outdoor public space, infrastructure, equipment, or vehicle that is not accessible to persons with disabilities;

(b) information, communication, and technological barriers, including any information, communication, or digital technology that is not accessible to persons with disabilities, including by reason of the absence of alternative formats, the absence of plain-language presentation, the absence of accommodation for non-print readers, or the absence of compatibility with assistive technology;

(c) attitudinal barriers, including any belief, assumption, stereotype, or prejudice held by an individual, an organization, or a system that operates to limit the full and equal participation of persons with disabilities;

(d) policy and practice barriers, including any policy, procedure, regulation, rule, criterion, qualification, or practice that, in operation, hinders the full and equal participation of persons with disabilities, regardless of whether the policy, procedure, regulation, rule, criterion, qualification, or practice was adopted with discriminatory intent;

(e) systemic and structural barriers, including any pattern of policy, practice, funding, staffing, organizational design, or institutional arrangement that, taken as a whole, produces disproportionately negative outcomes for persons with disabilities, regardless of whether any individual element of the pattern was adopted with discriminatory intent.

The proposed definition has three substantive characteristics that distinguish it from the comparator definitions and that respond to the documented Alberta record.

Five-category enumeration. The definition enumerates five categories of barrier rather than the more compressed three- or four-category enumerations the comparator legislation typically uses. The five-category enumeration ensures that the substantive coverage extends across the full range of obstacles documented in the disability-rights and disability-policy literature. The categories are not mutually exclusive; many specific barriers fall within multiple categories simultaneously. The enumeration's purpose is to ensure that no substantial category of barrier is overlooked or excluded from the legislation's substantive coverage.

Explicit recognition of systemic and structural barriers. Category (e) explicitly recognizes systemic and structural barriers as a distinct category of barrier the legislation regulates. The recognition is operationally significant. The engineered repudiation documented in Part One Section 1.3a is a systemic and structural barrier in the substantive sense Category (e) addresses: a coordinated pattern of funding, staffing, organizational design, and policy that, taken as a whole, produces disproportionately negative

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

outcomes for children with disabilities in the Alberta K-12 system. None of the comparator legislation explicitly recognizes systemic and structural barriers as a distinct category. The comparator approach has been to address systemic and structural barriers indirectly through the policy-and-practice category and through the standards-development process. The Alberta legislation, on the documented Alberta record, must address systemic and structural barriers directly. The Category (e) recognition is the operational expression of the design philosophy's commitment to systemic transformation.

Disconnection from discriminatory intent. Categories (d) and (e) explicitly establish that the legislation's protection against barriers operates regardless of whether the barriers were adopted with discriminatory intent. The substantive principle is dispositive: the legislation's protective function is to address operational outcomes, not to assess intent. The intent question is the question that the *Alberta Human Rights Act* and analogous statutes are oriented toward. The legislation's accessibility-standards function is operationally distinct: it is to ensure that operational outcomes deliver substantive equality, regardless of how the policies and practices producing the outcomes were originally adopted. The disconnection from intent ensures that the legislation cannot be defeated by a defence that the barrier was not adopted with discriminatory purpose. Where the operational outcome is the disproportionately negative outcome, the legislation operates against the barrier, regardless of intent.

The proposed barrier definition is, on these three substantive characteristics, more comprehensive than the comparator definitions and more directly responsive to the documented Alberta record. The five-category enumeration ensures comprehensive substantive coverage; the explicit systemic-and-structural recognition ensures the engineered patterns documented in Part One are addressed at the definitional level; the disconnection from intent ensures the legislation operates against operational outcomes regardless of policy history.

Scope and Coverage

The legislation must specify which organizations are subject to its substantive standards and on what implementation timelines. The specification is the operational mechanism through which the legislation's reach is defined. The substantive principle is that the legislation should reach as broadly as the design philosophy and the comparator record support, with appropriate accommodations for small organizations and for Indigenous governance arrangements. The proposed coverage architecture has four components.

Provincial public sector. The legislation must apply, with the most demanding compliance obligations and the shortest implementation timelines, to the provincial public sector. The provincial public sector includes the Government of Alberta and its ministries, Alberta Health Services and its operating subsidiaries, Crown corporations and Crown agencies, school divisions (public, separate, francophone, charter), publicly funded post-secondary institutions, library boards, regional health authorities and their successors, provincial regulatory bodies operating under provincial-statute authority (the Law Society of Alberta, the College of Physicians and Surgeons of Alberta, the College of Registered Nurses of Alberta, and analogous bodies), and provincial-government-administered entities operating under provincial-statute authority. Municipal governments are covered, with appropriate engagement on the substantive scope and implementation timeline, recognizing the documented Alberta municipal-coalition support for accessibility legislation (the campaign producing this blueprint has documented Notices of

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

Motion or analogous endorsements from Calgary, Edmonton, Lethbridge, Camrose, and Red Deer). The substantive purpose of the broad public-sector coverage is to ensure that the provincial-government-administered architecture leads the implementation of the legislation's substantive standards, modelling the operational behaviour the legislation will subsequently require of private-sector and non-profit organizations.

Private sector. The legislation must apply to private-sector organizations operating in Alberta and falling within provincial jurisdiction (federally regulated employers and federally regulated industries are excluded; the federal *Accessible Canada Act* applies to those organizations). The private-sector coverage must operate on a tiered basis to address the operational reality that small businesses cannot bear the same compliance burden as large enterprises. The proposed tiered approach is as follows. Large private-sector organizations (those with 100 or more employees, or annual revenue above a defined threshold to be specified in regulation) are subject to the most demanding compliance obligations and the standard implementation timelines. Medium private-sector organizations (those with 50 to 99 employees, or annual revenue within a defined intermediate range) are subject to the most demanding obligations on extended implementation timelines, with operational support from the provincial implementation agency. Small private-sector organizations (those with fewer than 50 employees, and annual revenue below a defined threshold) are subject to a substantively reduced set of obligations focused on customer service and design of services, with extended implementation timelines and substantial operational support from the provincial implementation agency. The tiered approach mirrors the operational practice of the federal *Accessible Canada Act* and the comparator provincial statutes, calibrated to Alberta operational conditions.

Non-profit and charitable sector. The legislation must apply to non-profit and charitable organizations operating in Alberta and providing services to the public. The non-profit coverage must also operate on a tiered basis. Large non-profit organizations (those above a defined size threshold) are subject to the standard obligations. Small non-profit organizations are subject to the reduced obligations on extended timelines. The non-profit-sector coverage is operationally significant because much of the disability-services delivery in Alberta operates through non-profit organizations contracted by provincial-government programs. The non-profit organizations that deliver disability services must, on the substantive principle that the legislation's protections apply to the full range of organizations persons with disabilities encounter, themselves comply with the legislation's substantive standards.

Indigenous-governance accommodation. The legislation must accommodate Indigenous-governance arrangements through a voluntary-participation and partnership framework rather than through direct provincial-government regulation of Indigenous governance entities. The framework recognizes that First Nations governance, Métis Settlement governance, and Indigenous-organization governance operate within constitutional and treaty frameworks that the provincial-government's regulatory authority does not override. The substantive provisions accordingly establish that Indigenous-governance entities are not directly regulated by the legislation but are eligible to participate voluntarily in the substantive-standards architecture, are eligible to receive operational support from the provincial implementation agency, and are eligible to enter into partnership agreements with the provincial government for the cooperative development and implementation of accessibility standards on terms that respect Indigenous-governance authority. The framework's substantive purpose is to ensure that Indigenous-governance authority is

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

respected while operational pathways exist for cooperative work between provincial-government accessibility infrastructure and Indigenous-governance entities. The framework is developed in detail in Section 3.6.

The four-component coverage architecture is operationally workable. The provincial public sector leads. The private sector follows on a tiered basis calibrated to organizational size and capacity. The non-profit sector operates on a parallel tiered basis. Indigenous governance is accommodated through voluntary participation and partnership. The architecture covers approximately the same operational space as the federal *Accessible Canada Act* and the comparator provincial statutes, with the tiered approach addressing the operational concerns about small-business compliance burden that have been raised in Alberta political discussion of accessibility legislation. The coverage architecture does not exempt small businesses from the legislation entirely; the substantive principle is that all organizations operating in the public realm must operate within accessibility standards, with the obligations calibrated to the organization's capacity to deliver compliance.

The Eight Substantive-Standards Areas

The legislation establishes eight substantive-standards areas within which accessibility standards are developed and operationally implemented. The eight areas are the operational domains within which the legislation's substantive coverage is structured. The substantive content of each area is developed in subsequent sections of Part Three (the seven non-education areas in Section 3.3; the education area in Section 3.4). Section 3.2 identifies the eight areas and specifies the substantive scope of each at a high level.

Area 1: Built environment and outdoor public spaces. Standards regulating the accessibility of buildings, infrastructure, outdoor public spaces, parks, playgrounds, recreational facilities, and the equipment associated with these settings. The area extends beyond the existing *Alberta Building Code* coverage, which addresses building accessibility at the time of construction or major renovation, to include the accessibility of existing buildings on a structured retrofit timeline, the accessibility of outdoor public spaces (which the Building Code does not cover), and the accessibility of equipment and facilities within the built environment.

Area 2: Information and communications. Standards regulating the accessibility of information, communications, and digital technology. The area includes digital-accessibility standards (Web Content Accessibility Guidelines version 2.1 Level AA as the substantive minimum, advancing toward 2.2 AA as the international standard evolves), document-accessibility standards (alternative formats, plain language, large print, Braille, accessible electronic documents), communication-accessibility standards (sign-language interpretation, augmentative and alternative communication, captioning, audio description), and technology-accessibility standards (compatibility with assistive technology, accessible-design requirements for technology procurement).

Area 3: Employment. Standards regulating accessibility in employment relationships, including recruitment, hiring, accommodation, retention, advancement, return-to-work, and termination. The area addresses both the accessibility of the employment relationship itself (accessible job-application

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

processes, accommodation in the hiring process, accommodation in the workplace) and the accessibility of employment-related systems (workplace policies, performance-management processes, return-to-work programs, leave provisions). The substantive content recognizes the documented Alberta employment record, including the systemic-exclusion documentation in the campaign's earlier work.

Area 4: Transportation. Standards regulating the accessibility of public transit, paratransit, intercity transit, taxi and rideshare services, transportation infrastructure (stations, terminals, bus stops), and transportation-related communications and information. The area covers transportation services operating within provincial jurisdiction; federally regulated transportation (interprovincial rail, federally regulated air, ferry services on federal waters) is regulated by the federal *Accessible Canada Act* and is excluded from provincial coverage but addressed through coordination provisions.

Area 5: Design and delivery of services and programs. Standards regulating the accessibility of services and programs, including customer service, service-design, program-design, and service-delivery practices. The area applies broadly across public-sector, private-sector, and non-profit-sector organizations. The substantive content draws on the customer-service-standard tradition the Ontario AODA and the Manitoba AMA established and extends to the design-and-delivery-of-services area on the federal *Accessible Canada Act* model.

Area 6: Education. Standards regulating the accessibility of K-12 education, post-secondary education, vocational training, adult education, and lifelong learning. The substantive content of the education area is developed in detail in Section 3.4 and operationalizes Article 24 of the United Nations *Convention on the Rights of Persons with Disabilities* within Alberta provincial jurisdiction. The education area addresses the engineered repudiation documented in Sections 1.3a and 2.7a: the funding-floor and EA staffing-ratio provisions, the segregated-setting transition pathway, the seclusion-room prohibition, the inclusive-education baseline, the consultation-requirement provisions, and the public-record reporting cycle.

Area 7: Health. Standards regulating the accessibility of healthcare facilities, healthcare services, healthcare communications, and healthcare-decision-making. The area addresses the accessibility of physical healthcare facilities, the accessibility of healthcare-related information (including informed-consent processes), the accessibility of healthcare-decision-making (including supported decision-making frameworks), and the structural capacity of the healthcare system to serve persons with disabilities (including healthcare-worker training in disability-affirming practice). The substantive content draws on the documented Alberta record of healthcare-access barriers and on Article 25 of the United Nations *Convention on the Rights of Persons with Disabilities*.

Area 8: Goods and procurement. Standards regulating the accessibility of consumer goods (where the goods are designed for use by persons with disabilities or where their accessibility affects persons with disabilities) and the accessibility-related requirements for public-sector procurement. The procurement-standard component is operationally significant: provincial-government procurement is a substantial leverage point through which accessibility requirements can be extended into the broader market. Provincial procurement specifications that require accessibility compliance produce private-sector accessibility behaviour even where the private-sector organizations are not directly regulated by the legislation's other substantive standards.

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

The eight areas are mutually reinforcing rather than mutually exclusive. Many specific accessibility issues operate across multiple areas simultaneously (a healthcare facility's built-environment accessibility, information-and-communications accessibility, and design-of-services accessibility all operate together). The substantive-standards-development process must accordingly address the operational interfaces between areas as well as the substantive content within each area.

What Section 3.2 Establishes

The substantive ground of the *Alberta Accessibility and Inclusion Act* is established through three definitional and structural choices and through the identification of eight substantive-standards areas.

The definition of disability operates within the social-model framework: the disability is in the interaction between the impairment and the barrier, not in the impairment alone. The definition explicitly enumerates physical, mental, intellectual, cognitive, learning, communication, and sensory impairment categories, and explicitly includes chronic illness, functional limitation, episodic and fluctuating conditions, progressive conditions, and invisible disabilities. The definition operates independently of the eligibility definitions used in Alberta's categorical-program architecture. The substantive purpose is to ensure that the legislation's protective function operates at the broadest substantive scope consistent with the international human rights framework.

The definition of barrier operates through a five-category enumeration that addresses physical-and-architectural barriers, information-and-communication-and-technological barriers, attitudinal barriers, policy-and-practice barriers, and systemic-and-structural barriers. The systemic-and-structural category is novel relative to comparator legislation and addresses the engineered patterns documented in Part One Section 1.3a directly. The legislation's protection against barriers operates regardless of whether the barriers were adopted with discriminatory intent; the substantive standard is the operational outcome.

The coverage architecture extends the legislation across the provincial public sector (with the most demanding obligations and the shortest timelines), the private sector on a tiered basis (large, medium, and small organizations with calibrated obligations and timelines), the non-profit and charitable sector on a parallel tiered basis, and Indigenous-governance entities through a voluntary-participation and partnership framework. Federally regulated organizations are excluded under federal jurisdiction and addressed through coordination provisions in Section 3.8.

The eight substantive-standards areas are: built environment and outdoor public spaces; information and communications; employment; transportation; design and delivery of services and programs; education; health; and goods and procurement. The substantive content of areas 1, 2, 3, 4, 5, 7, and 8 is developed in summary in Section 3.3. The substantive content of area 6 (education) is developed in detail in Section 3.4.

The substantive ground established in Section 3.2 is the foundation on which Sections 3.3 through 3.9 build. The definitions determine who the legislation protects and what it protects against; the coverage architecture determines which organizations are bound by the protections; the eight substantive-standards areas determine the operational domains within which the substantive content is developed. The next

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

section turns to the substantive content of seven of the eight areas, with education developed separately in Section 3.4.